HCC23/26

Ref Case: HCCR747/26

THE STATE

Versus

SIMON JACKSON

HIGH COURT OF ZIMBABWE
BACHI MZAWAZI J
CHINHOYI, 12 MAY 2026

M.M Chitsiga for the State
T Chitambira for the Accused.

Assessors: *1. Mr. Mutayiwa*
2. Dr. Mashavave

Criminal trial

BACHI MZAWAZI J: The accused person has a mental history predating the commission of this offence of murder, as it is defined in s47 of the Criminal Law (Codification and Reform) Act[1]. It is apparent that this condition necessitated pre-trial incarceration in terms of the Mental Health Act [2]as dutifully imposed by the Magistrates Court on initial remand. This court has learnt that the accused has been in custody ever since, a good four years after the commission of the offence.

Though under the relevant medication and mental evaluation, he had been remanded not into a mental institution but to an ordinary prison cell among sane inmates. In a way this integration, as per the dictates of the Declaration on Rights of Mentally Retarded Persons, proclaimed by the General Assembly Resolution 2856 (XXVI) of 20 December 1971, Article 4, promotes the mentally challenged person's right to live in circumstances as close as possible to a

[1] [Chapter 9:23]
[2] [Chapter 15:12] section 27

normal life with normal people though in a confined environment.[3] What is of concern, however, is the duration of the stay of similarly circumstanced persons, the mentally handicapped, in custody pending trial. In this case it took four years of continuous waiting and uncertainty, whilst on a daily basis, the rest of the inmates faced their trials or appeared on routine remands. This introductory summation highlights the dilemma this court was then faced with when sentencing the accused.

Given the accused's mental state at the time of the commission of the offence, the matter was brought before this court as a stated case. The statement of agreed facts revealed a bizarre incident where the accused strangled his then wife, the deceased to death. An altercation between the two had arose after a beer drink. The accused then concealed the deed by burying the deceased's body in a shallow grave in the middle of a field. He was at the time, cunny enough to report the deceased missing and to translocate to a different area. The remains were discovered a few days later in an advanced state of decomposition because of the shallowness of the grave. This led to the arrest of the accused person and the subsequent committal in terms of the Mental Health Act.

For a moment, from the accused's actions post the tragic incident of 13 March 2022, one cannot be blamed to conclude that these were carefully calculated actions of a sober minded person. That is, the digging of a grave in a hidden area, the burying of the body and then the lie as to the whereabouts of the deceased. What is important, however, is the accused's state of mind at the time of the strangling of the deceased. Mentally disturbed persons have their fair share of snapping and lucid moments, intervals or episodes.[4]

From the medical evidence that was placed on record, the psychiatric report stated that the accused was mentally incapacitated at the time of the commission of the offence. His examination

[3] Mentally challenged persons generally fall under the rubric of the UN Declaration on the Rights of Disabled Persons.

[4] Machona HH14/02, S v Patricia Makurumure HH46/12 Nkosiyabo Ndzombane v the State SC77/14.

revealed that due to a chronic disease which was treated late, the accused developed mental illness. This condition was further exacerbated by his abuse of prohibited drugs and other illicit substances.

In addition, *Mr Chitambira,* the assigned pro-deo counsel for the accused had difficulties in interviewing the accused. In his view, the accused was still very much mentally unstable although under strict medication and prison authority supervision. The trial prosecutor, *Ms Chitsiga,* shared the same sentiments during the case management session after seeing the accused. It is against this combination of factors that the legal counsels agreed to a statement of agreed facts.

The court was convinced that the accused person appeared to be abnormal when brought into the dock. An enquiry had to be conducted to ascertain whether he appreciated what was taking place and was fit to stand trial. The prison officer confirmed that the reason why the accused was acutely disoriented earlier that morning is because he had delayed in taking his prescribed medication. He stabilised soon after doing so and just in time for the court session.

On the evidence placed on record, against the statement of agreed facts, we are satisfied that the accused cannot be held liable for his actions because of insanity. In terms of the Mental Health Act, the accused person cannot be found guilty of murder per se, when he did not appreciate what he was doing at the time of the commission of the offence[5]. We accordingly, find the accused not guilty because of insanity. A special verdict is returned.

On sentencing the accused, the court took note of the preliminary observations highlighted earlier on, as well as the psychiatrist's recommendation that the accused be released in the custody of his brother. No relative of the accused had been informed of the trial date. In light of the need to recognise the rights of mentally challenged persons to live a life close to normalcy as possible, this necessitated a further postponement to invite the accused's relatives. The court had to be

[5] S29(2)(a) of the Mental Health Act [Chapter i5:12] s227/s229 of the Criminal law Codification and Reform Act [Chapter 9:23] S v K A Juvenile HH49/09.

assured that the person mentioned as guardian, primary care giver or any other person prepared to assume those roles, would be able to monitor the accused's intake of his medication.

Unfortunately, from Ms *Chitsiga's* report, no relative of the accused featured. Further, from those she interviewed, no one was interested in coming to court or accommodating the accused. In the absence of a responsible person, the danger posed is that the accused person may not have access to the requisite medication and that he may not adhere to the stipulated times for taking the medication.

In terms of the governing law, once a special verdict has been returned, the accused is a free man entitled to his liberty. The sentencing court has one of two choices, to release him in the custody of a responsible relation, who sees to it that there is strict adherence to the requisite drug regime or to commit him to an institution for further management and evaluation in terms of the Mental Health Act.[6]

In *casu*, the only available option, given that the accused did not appear to be much in control of his mental faculties when not under medication, is to begrudgingly order for a further institutionalisation. Institutionalisation, at this juncture means constant, supervised medical care until fit enough to be released into the public domain. This, in the interim, will protect both the interests of the public and safeguard the accused from relapse and recidivism.[7]

In order to avert, the danger of yet another prolonged stay in prison pending the said examination the court will give timelines to enable speedy reintegration of the accused into society in observance of his right to be assisted to live a life as close to normalcy as can be.[8]

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[6] *Section 29 of the Mental Health Act [Chapter 15:12]*

[7] *See, S v Lameck Charidza* HMA10/17 where the court shared the same sentiments.

[8] See Declaration on the Rights of Mentally retarded persons, proclaimed by the General Assembly Resolution 28569 (XXVI) of 20 December 1971

Accordingly, it is hereby ordered that, the accused be transferred from Chinhoyi Prison to Chikurubi Prison Psychiatric Unit for further mental evaluation and treatment in terms of the Mental Health Act. The accused is to be treated and certified by the relevant authorities as stipulated in s29 (4) and (7) of the Mental Health Act, within a period of not more than 18 months of this order. He should not be detained for a period exceeding 18 months from the date of this order. [9]

National Prosecuting Authority, State's Legal Practitioners.
Mangwiro Law Chambers, Accused's Legal Practitioners

[9] See The *State v Jealous Tomasi* HCC 2/25